

Implementation plan

Implementation Plan: Multidisciplinary Team (MDT) Integrated Rehabilitation Model
For Kon Tum Social Work Center – Inclusion 2b Project
Enhancing Quality of Life for Persons with Disabilities (PWD) in Agent Orange-affected provinces

1. Strategic Framework and Design Philosophy

The current rehabilitation landscape in Vietnamese social protection centers is characterized by clinical silos, fragmented "session-only" services, and limited coordination between rehabilitation therapists, teachers, caregivers, and social workers. This results in suboptimal functional outcomes, inconsistent care plans, reduced independence in Activities of Daily Living (ADL), and limited social participation for PWD, particularly those affected by Agent Orange-related disabilities.

This Implementation Plan mandates a systemic shift from parallel, discipline-specific care to a **unified, function-oriented, interdisciplinary architecture**. The central objective is to place the PWD and their family at the core of service delivery, transforming the center into a platform for real-world autonomy, social inclusion, and improved quality of life (QoL).

Core Design Principles (updated with stronger person-centered focus):

Principle	Description	Intended Impact on PWD Outcomes
Person-centered	"One PWD = One Plan" with active PWD and family voice in all stages.	Ensures goals reflect individual/family priorities; reduces conflicting advice and empowers families.
Function-oriented	Prioritizes real-life mobility, communication, ADL independence, and social	Directly improves functional independence and QoL.

Principle	Description	Intended Impact on PWD Outcomes
	participation over isolated clinical metrics.	
Integration-focused	Embeds rehabilitation into every routine, classroom activity, and daily interaction.	Maximizes "functional practice" across all waking hours.
Interdisciplinary	Promotes shared goal-setting, joint problem-solving, and overlapping training among disciplines.	Enhances synergy, reduces fragmentation, and improves holistic outcomes.

Evidence Base: Multidisciplinary and interdisciplinary approaches have demonstrated improvements in functional status, ADL performance, reduced hospitalizations, and better social integration in rehabilitation settings. In resource-limited contexts like Vietnam, embedding rehab into daily care and education is particularly effective for sustainable gains.

Vietnamese Contextual Adaptation: The model aligns with Vietnam's Law on Persons with Disabilities (2010), national social protection policies, and MOLISA frameworks. It addresses local realities such as staff capacity constraints, family caregiving burdens (often shouldered by women), cultural emphasis on family involvement, and administrative coordination challenges in provincial centers.

2. The MDT Architecture: Roles and Responsibilities

Success depends on clear roles, strong coordination, and behavioral change support. The model introduces a **core MDT** (daily operations) and an **extended MDT** (for complex cases, e.g., speech/communication or mental health needs).

Key Roles:

- **Rehabilitation Therapist:** Technical lead for functional assessments and motor/communication strategies. Provides "how-to" guidance and supports embedding techniques.
- **Teacher (Special/Inclusive Education):** Embeds functional goals into cognitive, learning, and social activities. Ensures education reinforces rehabilitation.

- **Caregiver:** "Co-therapist" role — implements rehab actions during ADL routines (meals, dressing, hygiene). Provides daily feedback.
- **Social Worker (Case Manager): Systemic Linchpin** and administrative owner of the Integrated Individual Plan (IIP). Facilitates coordination, family engagement, weekly meetings, and resolves conflicts.
- **System Architect (Process Engineer):** Temporary/consultative role (reporting to Center Management) responsible for SOP integrity, staff training on workflows, behavioral change management, and preventing reversion to silos during rollout.

Responsibility Matrix (RACI – Responsible, Accountable, Consulted, Informed):

- **IIP Ownership:** Social Worker (Accountable); all disciplines (Responsible for their sections).
- **Daily Integration:** Caregivers & Teachers (Responsible); Therapist & Case Manager (Consulted).
- **Weekly Review:** Case Manager (Accountable); full MDT (Responsible).
- **Family Engagement:** Case Manager leads; all staff support.

Five Essential Staff Competencies (with added emphasis on interdisciplinary skills):

1. Rehabilitation science & functional therapy.
2. Special/inclusive education & goal linking.
3. Case management & person-centered planning.
4. Systems design & SOP adherence.
5. Vietnamese context (family dynamics, local policies, low-resource adaptation).

Interdisciplinary Practices: Joint assessment sessions, shared documentation, co-training sessions (e.g., therapists training caregivers on techniques), and regular joint problem-solving.

3. The Service Pathway: From Intake to Integrated Care

The pathway forms a continuous improvement loop rather than linear hand-offs.

Standard Service Flow:

1. **Intake & Assignment** — Initial screening + Case Manager assigned within 3–5 days.
2. **360-Degree Multidisciplinary Assessment** — Involves all core MDT members + family input.
3. **IIP Co-Development** — MDT meeting with PWD/family participation to create unified plan.
4. **Daily Implementation** — Rehab embedded in care routines and education (SOP-04).
5. **Ongoing Monitoring & Logging** — Daily/weekly logs (SOP-06).
6. **Weekly MDT Review & Adjustment** — Data-driven plan updates (SOP-05/07).
7. **Periodic Evaluation** — 4-week and quarterly outcome reviews.

Multidisciplinary Assessment (enhanced with family column):

Discipline	Focus Area	Key Data Points	Family Input Required
Therapist	Physical & Functional	Mobility, fine/gross motor, communication	Observed home routines & barriers
Teacher	Cognitive & Learning	Attention, learning style, social behavior	Child's interests & family expectations
Caregiver	ADL Independence	Eating, dressing, hygiene, mobility	Caregiver capacity & daily challenges
Social Worker	Social & Contextual	Family capacity, economic situation, support networks	PWD/family vision & priorities

4. The Integrated Individual Plan (IIP) Framework

The IIP is the **single source of truth** — a living, person-centered document signed by PWD/family and MDT.

IIP Template Structure (expanded):

- **Section 1:** Basic Information (PWD ID, diagnosis, Case Manager, date).
- **Section 2:** Multidisciplinary Assessment Summary (including family voice).

- **Section 3:** Goal Setting (using **SMART** format: Specific, Measurable, Achievable, Relevant, Time-bound).
 - Functional goals (e.g., “PWD will independently self-feed a full meal using adapted utensils in 4 weeks”).
 - Education goals linked to functional skills.
 - Daily Living goals.
 - Long-term participation goals (social inclusion).
- **Section 4:** Intervention Plan & Role Assignment (who, what, when, where, how).
- **Section 5:** Daily Integration Log (caregiver/teacher tracking).
- **Section 6:** Weekly MDT Review Notes (progress, barriers, adjustments).
- **Section 7:** Family/Caregiver Capacity & Support Plan.
- **Section 8:** Outcome Measurement & Review Schedule (with signatures).

Goal Linking Logic: Every education or care activity must explicitly connect to at least one functional goal (e.g., writing practice → fine motor control; group play → social communication).

PWD/Family Voice: Mandatory visioning session during IIP development. Goals written in accessible language reflecting the person’s own words where possible.

5. Integration Mechanics: Rehabilitation in Daily Life and Education

Philosophy: Rehabilitation is not a separate session — it is embedded in the other 23 hours of the day through guided, intentional practice.

Daily Care Integration Matrix (examples):

Routine	Embedded Rehab Action	Functional Target	Responsible Staff
Eating	Hand coordination, grip strengthening, sequencing	Independent self-feeding	Caregiver + Therapist guidance
Dressing	Motor planning, balance, sequence recall	Personal autonomy	Caregiver

Routine	Embedded Rehab Action	Functional Target	Responsible Staff
Hygiene	Weight-shifting, balance, reach training	Fall prevention & mobility	Caregiver

Education Integration Matrix (special education focus):

Learning Activity	Embedded Rehab Goal	Functional Target	Responsible Staff
Writing/Drawing	Fine motor control & hand strength	Functional literacy & communication	Teacher + Therapist
Group Work	Social interaction, turn-taking, communication cues	Social participation	Teacher
Physical Activity	Gross motor coordination & stamina	Physical mobility & endurance	Teacher + Therapist

Behavioral Mandates:

1. All activities must link to an IIP goal.
2. Caregivers act as co-therapists (guided support, not full assistance).
3. Staff use simple prompts/scripts for consistent embedding.
4. Prioritize PWD participation and choice in routines.

6. The SOP System: Operationalizing the Model

The SOP package ensures consistency despite staff turnover. Each SOP includes: Purpose, Scope, Responsibilities, Step-by-Step Procedure, Templates/Forms, KPIs, Training Requirements, and Revision History.

Core SOPs (refined and expanded):

- SOP-01–03: Intake, Assessment, IIP Development (led by Case Manager).
- SOP-04: Daily Implementation & Integration (checklists for logs).
- SOP-05: Weekly MDT Coordination Meeting (standard agenda template).
- SOP-06: Monitoring & Daily Logging (simple, low-burden forms).

- SOP-07: Review, Adjustment & Outcome Evaluation.
- SOP-08: Quality Assurance, Supervision & Continuous Improvement.

Additional Tools in SOP Package:

- Standardized forms (IIP template, daily log, meeting minutes, assessment checklist).
- Quick-reference job aids and visual flowcharts.
- Data privacy protocols for IIP documentation.

7. Monitoring, Evaluation, and Quality Assurance

Key Performance Indicators (KPIs):

- 100% of PWD have a current, signed IIP.
- ≥90% MDT meeting attendance and data-backed discussions.
- Documented integration in ≥80% of daily logs.
- Functional progress (e.g., % of SMART goals achieved; simple ADL scales or observed independence levels).
- Family/caregiver satisfaction and capacity improvement.
- Reduced regression or unplanned transfers.

Tools: Daily logs feed weekly reviews; quarterly reports link to project QoL goals. Use simple pre/post observations or validated low-resource functional measures where feasible.

Risk Mitigation:

- Staff turnover → Cross-training and peer mentoring.
- Resistance to change → Change management workshops led by System Architect.
- Resource constraints → Leverage existing routines; train-the-trainer model.
- Family non-engagement → Culturally sensitive outreach and support plans.

8. Implementation Roadmap and Phased Execution

10-Week Pilot Preparation Timeline (aligned with TOR deliverables):

Phase	Timeline	Key Activities	Critical Outputs
1: Inception & Mapping	Weeks 1–2	Desk review, stakeholder consultations, gap analysis	Current-state report; methodology
2: Model Customization	Weeks 3–4	Adapt MDT structure, visual diagram, roles	MDT Model document + organigram/flowchart
3: SOP & Tool Development	Weeks 4–6	Draft 8 SOPs, IIP templates, integration tools	Complete SOP package + training toolkit
4: Capacity Building	Weeks 5–7	Staff training (Goal Linking, Case Management, family engagement)	Trained core MDT team
5: Validation & Testing	Weeks 8–10	Real-case simulations (physical & intellectual disability profiles), stress-testing	Pilot-ready package; validation report

Post-Pilot Sustainability (new addition for long-term results):

- 3–6 months of ongoing supervision and mentoring.
- Train-the-trainer program for internal champions.
- Scaling strategy to other centers in Gia Lai/Quang Ngai.

Case-Based Design: Develop and test using 3–5 real PWD cases during validation.

Visual Elements (to be included in final deliverables):

- MDT organigram showing coordination loops.
- Service pathway flowchart.
- Sample IIP template with filled examples.
- Integration matrices as job aids.

9. Pilot Readiness Success Checklist

- ☐ One PWD = One Plan (signed IIP for all pilot participants).
- ☐ Clear role definition and Case Manager empowerment.
- ☐ Evidence of daily integration in logs.
- ☐ Consistent weekly MDT meetings with full attendance.
- ☐ Family involvement documented and supported.
- ☐ Staff trained and demonstrating Goal Linking.
- ☐ Basic outcome data collection system in place.

This enhanced MDT Integrated Rehabilitation Model transforms fragmented services into a cohesive system that prioritizes functional independence, family partnership, and sustainable daily practice. By engineering coordination, embedding rehabilitation into every aspect of center life, and building local capacity, the model will deliver measurable improvements in ADL independence, social participation, and quality of life for PWD at Kon Tum Social Work Center and beyond.

Annexes (to be developed):

- Full SOP templates and forms.
- Training curriculum outline.
- Sample SMART goals and integration scripts.
- Visual diagrams and flowcharts.